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OPERATIONAL POLICY

Handover of Patient Care

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Key Points

- Effective handover of a patient's care is essential for the continuity, quality and safety of patient care.
- Consider anaesthetic follow up review for High risk patients returning to the ward

Introduction

Effective handover of a patient's care is essential for the continuity, quality and safety of patient care.

Systems exist for a standardised transfer of information between healthcare professionals.

Conduct of Handover

During handovers only one person should speak at a time, and the conversation during the handover should relate only to the patient. Non-handover activities should cease during the handover. Each team member should be given the opportunity to ask questions and clarify information.

Handover in Recovery or on Admission to Critical Care

It is essential that the anaesthetist formally hands over care of the patient to the practitioner assigned to receive the patient.

For recovery, utilise the handover communication tool displayed in each recovery bay to structure your handover.

It should include:

Patient details:

- Name and age
- Relevant comorbidities
- Drug history and allergies

Surgical details:

- Type of operation with the estimated blood loss (EBL)
- Antibiotics given
- Any complications or problems associated with the procedure

Anaesthetic details:

- The basic technique used
- The analgesia and anti-emetics administered
- Fluids and blood products administered during the operation

- Any complications or problems associated with the Anaesthetic

Post-Operative Plan:

- Make a clear management plan for the immediate post-operative period. Highlight any concerns you have and document who to contact in case of any problems. I**nclude:**
    - Oxygen and monitoring requirements (Document clearly if Intrathecal Diamorphine has been given)
    - Analgesia, anti-emetics and fluids regimes
    - Acceptable physiological parameters (e.g. blood pressure)

Before you leave recovery or critical care, you must:

- Document your plan clearly.
- Ensure that you have completed your anaesthetic record and drug chart.
- Check your patient is stable and your recovery nurse is happy before leaving the recovery room.

Handover of care between Anaesthetists during Surgery

A comprehensive handover should occur whenever an Anaesthetist is taking over the intra-operative care of a patient.

Handover of Care by On Call Anaesthetist (063)

The 063 Anaesthetist should use the Handover sheet provided in theatre reception. (See attached)

Post Operative High Risk patients returning to the Ward

The following patients should be included in the handover for ward review by a member of the anaesthetic team within 24 hours of their operation:

- Upon request by the List Anaesthetist
- ASA 4 or 5;
- Mortality Risk ≥5%
- Clinical Frailty score ≥5

Obstetric Handover

All anaesthetist to anaesthetist obstetric shift handovers must occur in the Handover Room on Labour Ward, using the patient information boards. This is to ensure we are communicating the latest up to date information available on our obstetric patients i.e. from the labour ward patient information boards. Please be aware that midwifery handover also occurs in the Handover Room on Labour Ward around the same time as anaesthetist handovers.

References

- AAGBI Safety Guideline 2013: Immediate Post-Anaesthesia Recovery
- RCA Guide for Novice Trainees.